NCCP 00414 · VERSION 5A

Palbociclib with endocrine therapy

Take palbociclib for 21 days, then have 7 days without palbociclib. The aromatase-inhibitor tablet continues every day, or fulvestrant injections follow their separate schedule. The week off applies to palbociclib, not automatically to the hormone treatment.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Reference timing, not booked appointments. Ask who requests each test, when it is due, who reviews the result and whom to contact if it is missing. Delays can change cycle numbers; time-based monitoring remains time-based.

Check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Palbociclib and endocrine treatment	Confirm the partner and ovarian suppression if relevant	Palbociclib Days 1–21; rest Days 22–28	Repeat 28-day pattern if the team clears it	AI continues daily; fulvestrant follows its injection schedule	The team confirms each hold, dose change or stop; partner reviewed separately
FBC / neutrophils / platelets	Baseline FBC	Day 1 and Day 15; extra Day 22 test if needed	Cycle 2: Day 1 and Day 15; from cycle 3: Day 1	Only after ≤grade 2 neutropenia throughout first 6 cycles: may reduce to every 3 months before a cycle	Extra FBC for low counts, fever, infection or clinical concern
Kidney / liver bloods	Baseline renal and liver profile	Before cycle 1	Before each cycle	Before each cycle; additional testing as indicated	Review organ changes before a dose decision
ECG / echo	If indicated by symptoms, cardiac risk or another drug	No routine palbociclib-specific ECG/echo date	Investigate if clinically indicated	Do not copy the ribociclib ECG timetable	Reassess new cardiac symptoms
Bone health / DXA	Assess partner, ovarian suppression and fracture risk	Arrange the agreed partner-specific plan	Follow results and individual risk	Repeat DXA according to endocrine/bone guidance and risk	Review when endocrine treatment, suppression or fracture risk changes
Response imaging / markers	Record measurable sites and baseline imaging	Agree first response review	Usually every 2–4 months; individualise	Adapt to disease pace; earlier for symptoms	Markers supplement, never replace, assessment

THE WHOLE TREATMENT JOURNEY

Palbociclib with endocrine therapy: the whole pathway

HR-positive, HER2-negative locally advanced or metastatic breast cancer

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Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Treatment NCCP	Confirm exact indication and biomarkers	Usual palbociclib dose: 125 mg once daily on Days 1–21 of a 28-day cycle; reduced prescribed doses are 100 mg or 75 mg.	Take palbociclib for 21 days, then have 7 days without palbociclib. The aromatase-inhibitor tablet continues every day, or fulvestrant injections follow their separate schedule. The week off applies to palbociclib, not automatically to the hormone treatment.	Continue with safety / benefit review	Treatment continues while it is helping and side effects remain acceptable. There is no fixed adjuvant course in NCCP 00414. Blood results or symptoms may change the dose or postpone the next cycle.
Surgery / radiotherapy ESMO_MBC2023	Only for an agreed local-control or symptom need	Teams decide sequence / any treatment interruption	No routine surgery or RT implied	Review any local-treatment effects	Reassess if goals or symptoms change
Safety checks NCCP · SMPC	Baseline bloods and individual risks	Early checks from the monitoring table	Regimen-specific checks continue	Review results before continuation	Resolve abnormalities / agree next plan
Scans / follow-up ESMO_MBC2023	Confirm baseline and treatment setting	Agree the next imaging milestone	Use the matching surveillance / response plan	Review benefit and symptoms	Continue setting-specific follow-up

What each check is for

1 **Blood tests**

Before the first dose: FBC, kidney and liver bloods; review other medicines, pregnancy possibility, receptor results, menopausal status and the exact endocrine partner. Confirm ovarian suppression when relevant. FBC on Day 1 and Day 15 of cycles 1–2, then Day 1 from cycle 3. Kidney/liver bloods before each cycle. If counts are low, extra blood tests and a pause may be needed. Only after no worse than grade 1/2 neutropenia in the first 6 cycles may FBC frequency reduce to every 3 months before a cycle, with additional tests when clinically indicated.

Sources: NCCP · SMPC

2 **Heart checks**

Palbociclib does not have the routine ECG timetable used for ribociclib. ECG or echo is arranged if symptoms, another medicine or individual cardiac risk make it appropriate.

Sources: NCCP · SMPC

3 **Imaging**

Response assessment in advanced breast cancer is generally every two to four months, adapted to disease dynamics, sites, symptoms and treatment. Less frequent monitoring may suit indolent disease. New symptoms or suspected progression need earlier assessment. The team chooses the imaging method; this is not an automatic three-month CT chest/abdomen/pelvis order.

Sources: ESMO_MBC2023

4 **Bone health**

Bone-health monitoring belongs to the endocrine partner, ovarian suppression and personal fracture risk. If taking an aromatase inhibitor, agree the baseline bone-density assessment and follow-up plan with the team; palbociclib itself has no fixed DXA interval.

Sources: NCCP · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.